

AdvocateAuroraHealth

Title: AAH Business Continuity Plan		Document Number: 2437
Document Type: ☒ Policy ☒ Procedure ☐ Guideline ☐ Other		Last Review/Revision Date: 07/28/2021
Content Applies to Patient Care: (Select all that apply) ☐ Adults ☐ Pediatrics (Under 18)	Content Applies to: (Select One) ☐ Clinical ☒ Administrative	Next Review Date: 07/28/2024
		Effective Date: 07/28/2021
Scope: ☒ AAH System ☐ ACH ☐ AAH IL Only ☐ AAH WI Only ☐ Site Only (Location Name): ☐ Department Only (Department Name):		

I. PURPOSE

The purpose of the Business Continuity Plan (BCP) is to provide expectations to Advocate Aurora leadership when an unplanned incident occurs that may alter or disrupt normal business functions.

This procedure may work in tandem with the Advocate Aurora Health Informatics & Technology Disaster Recovery Policy providing an assessment and response to a business disruptive incident.

II. SCOPE

This policy and procedure applies to executive and system leadership from all departments and service lines for Advocate Aurora Health, Inc.

III. DEFINITIONS/ABBREVIATIONS

Aurora Administrative Office Security Command Center (AAOASCC) – 24/7 dispatch center staffed by trained dispatchers providing continuous communications system-wide.

Business Continuity Plan (BCP) – Plan that provides direction for business continuance from a serious business disruption.

Business Impact Analysis (BIA) – Tool that identifies critical business functions and recovery strategies for a department or service-line.

Healthcare Information Technology (HIT) – Application of technology in the healthcare industry.

Hospital Incident Command System (HICS) – Standardized system to organize incident response personnel, resources and operations.

Incident Command System (ICS) – Central component to the National Incident Management System providing framework used nationwide for incident management.

Incident Management Team (IMT) – Pre-determined positions convened to provide incident management support.

Quality Management Oversight Committee (QMOC) – System recognized committee that provides quality oversight.

Vice President (VP) – Organization department or service-line executive.

IV. POLICY

The Business Continuity Plan provides guidance to leadership when an unplanned incident occurs that may alter or disrupt normal organizational business functions.

A. Activation

The Business Continuity Plan (BCP) will be activated once an event has been identified that is taxing overall business operations. Triggers that may necessitate activation of the BCP include, but are not limited to:

1. Receiving notice of a credible threat leading the organization to enhance readiness posture and prepare to take necessary actions.
2. An unplanned event/emergency or disruption has occurred involving personnel, sites, equipment that will cause business disruption.
3. A situation necessitating evacuation of a geographic area causing or potentially causing business disruption.

B. Governance

Governance of the Business Continuity Plan rests with the Business Continuity Planning Team and Director of Emergency Preparedness. The Emergency Preparedness Council (comprised of the Emergency Preparedness Clinical Site Committee and Emergency Preparedness Corporate Committee) will serve as the Steering Committee and subject matter experts as needed.

C. Evaluation

1. A minimum of two exercises will be held annually to ensure the workability of the Business Continuity Plan. Exercises may be accomplished by workshops, table top exercises, full scale exercises or participating in Site exercises. Real-world events will fulfill one or both exercises.
2. Advocate Aurora Healthcare will utilize the Homeland Security Exercise and Evaluation (HSEEP) Preparedness Cycle in planning and evaluating all exercises.

3. All exercises will be tracked by the Emergency Preparedness Corporate Committee and will include an After-Action Report/Improvement Plan. The After-Action Report/Improvement Plan will be shared with the Advocate Aurora Health Quality Management Oversight Committee (QMOC).

D. Review of the Plan

1. The Advocate Aurora Business Continuity Planning Team/ Emergency Preparedness Corporate Committee and system Director of Emergency Preparedness will ensure the review of the Plans and Procedure.
 - a. Will review for updates to BCPs for all required regulatory elements
2. The Advocate Aurora Healthcare Business Continuity Plan and Procedure will be reviewed the first quarter annually. The Business Continuity Plans will be reviewed and updated annually according to the Business Continuity Planning Update Calendar.
3. Each department/service-line will be responsible for the review and updates of their respective Business Impact Analysis and plan.
4. The Plan is a living document and must be updated in real-time as organizational and/or departmental changes occur. The Procedure will be amended accordingly.
5. The policy will be reviewed annually.

V. PROCEDURE

A. Implementation Procedure

1. Upon receiving credible information of a potential or actual incident causing a business interruption, the Business Continuity Assessment Team is notified by the HIT Help Desk or the Aurora Administrative Office Security Command Center (AAOSCC). The Business Continuity Assessment Team is comprised of leadership team members from HIT / Disaster Recovery (DR) and Emergency Preparedness who evaluate the situation.
2. If a threat or event is deemed credible, the Business Continuity Assessment Team notifies the VP of HIT and VP of Security and Public Safety for further assessment of the event.
3. A senior leadership team may be convened to function as the Incident Management Team (IMT). The Hospital Incident Command System (HICS) is followed for the duration of the event. This team may be convened in person or virtually, this information is communicated by a Business Continuity Assessment Team member.

4. The IMT reviews the incident and determine if the Business Continuity Plan should be activated.
5. Depending on the incident, the IMT **may** consist of the listed positions or *designees*:
 - (a) Chief Operating Officer (COO)
 - (b) Chief Medical Officer (CMO)
 - (c) Chief Nursing Officer (CNO)
 - (d) Executive VP of Operations
 - (e) Chief Financial Officer (CFO)
 - (f) Chief Information Officer (CIO)
 - (g) Chief Marketing Officer
 - (h) Chief Human Resource Officer
 - (i) Chief Legal Officer
 - (j) Chief Spiritual Officer
6. Designees are defined as team members from the department/service-lines that are led by the Executive Leadership Team members. Activating department/service-lines are situation dependent and may not require the activation of all department/service lines.
7. Critical business partners, vendors or subcontractors may be informed of the incident or an unplanned downtime. The IMT team determines if a communication is necessary to inform any vendors or subcontractors, in accordance with any applicable regulatory requirements.

B. Operations/Critical Business Functions

1. Critical Business Functions are identified by department/service-lines in their respective Business Impact Analysis (BIA). These activities are performed continuously or resumed quickly following a disruption.
2. The Business Impact Analysis serves as a department's continuity plan that identifies their essential functions, staffing, vital records, and key applications, equipment, and supplies. Implementation of a department's BIA/continuity plan are based on the needs and considerations of the actual incident and resources available.
3. In the event of BCP activation – it is preferable to team members/authors listed on the BIA to help with the activation of the specific BIA. In the event team members are unavailable, the BIAs are written in a simple, understandable format identifying critical functions and resources needed.

C. Resume to Normal Operations

1. Returning to normal business operations may be dependent on the type and magnitude of the incident that occurred.
2. The IMT leads the process to return to normal operations. Referencing the department/service-line BIAs and the HIT Disaster Recovery Plan guides this process.

VI. CROSS REFERENCES

Not Applicable

VII. RESOURCES AND REFERENCES

- A. Assistant Secretary for Preparedness and Response (ASPR); Healthcare COOP & Recovery Planning. January 2015
- B. American Society for Industrial Security (ASIS); Business Continuity Management Systems; Requirements with Guidance for Use. 2010
- C. Harvard School of Public Health; Essential Functions and Considerations for Hospital Recovery. June 2014
- D. Federal Emergency Management Agency; Continuity Guidance Circular. February 2018
- E. Los Angeles County Emergency Medical Services Agency, Business Continuity Plan. February 2016
- F. National Fire Protection Association (NFPA) 1600. 201
- G. Payment card industry data security standards v.3.2.1 11.1.2, 12.5.3, 12.10
- H. <https://www.phe.gov/Preparedness/planning/authority/nhss/Documents/NHSS-Strategy-508.pdf>
- I. <https://www.cms.gov/files/document/updated-guidance-ma-and-part-d-plan-sponsors-42120.pdf>

VIII. ATTACHMENTS

Not Applicable